Referral packet F34: readable copy

Source: F34.pdf (fictional), 10 pages. Left side of each sheet is the original page, right side is a typed transcript. Black boxes are privacy redactions, not missing information.

Page map

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1	Fax cover sheet	fax_cover	good	
2	IHH Recuperative Care Referral Form	referral_form	good	
3	CalAIM Community Supports authorization request	calaim_auth	good	
4	Patient face sheet	face_sheet	good	
5	History and Physical - page 1	h_and_p	good	
6	History and Physical - page 2	h_and_p	good	
7	Gastroenterology consult and colonoscopy	consult_note	good	
8	Hospitalist progress note	progress_note	good	
9	Case management and social work note	social_work	good	
10	Vitals and labs	vitals	good	



FAX F34 FICTIONAL DEMO RECORD - NOT A REAL PATIENT PAGE 01/10

Fax cover sheet

Sunridge Community Hospital (fictional)
Case Management Department
FAX TRANSMITTAL
To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Gabriel Montoya, MSW
Phone: 555-0186
Date: 10/21/2026 10:20
Pages (including cover): 10
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]
Message
61F lower GI bleed s/p transfusion, Hgb stable.
Medically ready for d/c. Requesting recuperative care.
CONFIDENTIALITY NOTICE
This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 1: Fax cover sheet

fax_cover | orientation normal | legibility good

Sunridge Community Hospital (fictional)

Case Management Department

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Fax: 555-0199
From: Gabriel Montoya, MSW
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Re: Recuperative care referral
Urgent **[X]** For Review **[X]** Please Reply **[X]**

Message

61F lower GI bleed s/p transfusion, Hgb stable.
Medically ready for d/c. Requesting recuperative care.

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-- FICTIONAL DEMO RECORD - not a real patient --

FAX F34 FICTIONAL DEMO RECORD · NOT A REAL PATIENT

PAGE 02/10

ILLUMINATION Recuperative Care Referral Form

HEALTH • HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ithomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Sunridge Community Hospital (fictional) | Dept/Floor: 3 North Medical
SW/CM/RN: Gabriel Montoya, MSW | Phone/Pager #: 555-0186
Email: gmontoya@sunridgecommunity.example | Nursing Station #: 555-0187
Authorized by: Dr. A. Brandt | Phone #:

Patient Information

Patient Name: Ruth Doe | MRN: SR-2207461 | Date of Birth (dd/mm/yyy): 11/02/1964
Gender: ☐ Male ☒ Female ☐ Transgender ☐ Other | Ethnicity: White
Last 4 # of Social Security: xxx-xx- 0000 | Insurance: Orange Coast Health Plan (fictional) Medi-Cal | Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
Lower GI bleed, acute blood loss anemia
Admit Date: 10/17/2026 | Any Wounds? ☐ Yes ☒ No
If yes, explain the number, location, size and stage:

Independent with Wound Care? ☒ Yes ☐ No | If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 10/17/2026 Results: CXR clear

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* X No	Active Infection	Yes* X No
*Explain		*Explain	
Self Administer All Meds	X Yes No*	Ambulatory	X Yes No
Continent of Bowel & Bladder	X Yes No	Assistive Device	Yes* X No
Colostomy/Ileostomy	Yes X No	*Explain	
Foley Catheter	Yes X No	Hospice	Yes* X No
Diabetes	Yes X No	*Explain	
Insulin	Yes X No	Car	Yes X No
Diabetes related Oral Meds	Yes X No	Service Animal	Yes X No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? X No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain none

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

2871 PULLMAN ST., SANTA ANA, CA 92705 | (949) 273-0555 | ILLUMINATIONHEALTHHOME.ORG RC0725-SH

Page 2: IHH Recuperative Care Referral Form

referral_form | orientation normal | legibility good

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Sunridge Community Hospital (fictional)
Department/Floor: 3 North Medical
Social Worker/Case Manager/Nurse: Gabriel Montoya, MSW
Phone: 555-0186
Email: gmontoya@sunridgecommunity.example
Nursing Station: 555-0187
Authorized By: Dr. A. Brandt

Patient Information

Name: Ruth Doe
MRN: SR-2207461
Birth Date: 11/02/1964
Gender: F
Ethnicity: White
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused **[X]**
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Lower GI bleed, acute blood loss anemia
Admit Date: 10/17/2026
Any Wounds: Yes [ ] No **[X]**
Number/Location/Size/Stage:
Independent with Wound Care: Yes [ ] No [ ]
Home Health Ordered: Yes [ ] No **[X]**
PPD/TB Test or Chest X-Ray: Date: 10/17/2026 Result: CXR clear

History of Health & Living Situation

O2: Yes [ ] No **[X]**
Self Administer All Meds: Yes **[X]** No [ ]
Continent of Bowel & Bladder: Yes **[X]** No [ ]
Colostomy/Ileostomy: Yes [ ] No **[X]**
Foley Catheter: Yes [ ] No **[X]**
Diabetes: Yes [ ] No **[X]**
Insulin: Yes [ ] No **[X]**
Diabetes Oral Meds: Yes [ ] No **[X]**
Active Infection: Yes [ ] No **[X]**
Ambulatory: Yes **[X]** No [ ]
Assistive Device: Yes [ ] No **[X]**
Hospice: Yes [ ] No **[X]**
Car: Yes [ ] No **[X]**
Service Animal: Yes [ ] No **[X]**

Behavioral and Mental Health Status

Any limitations or challenges: No **[X]** Yes [ ]
Mental Health DX: Yes [ ] No **[X]** Type:
Non-compliant: Yes [ ] No **[X]**
Cognitive Impairment: Yes [ ] No **[X]**
Forgetful: Yes [ ] No **[X]**
Registered Sex Offender: Yes [ ] No **[X]**
Other: none

Length of Stay Authorized

Days: 14

Illumination Staff Only

Date Received:
Reviewed By:
Approved: Yes [ ] No [ ]
Date:
Authorization #:
Reason for Denial:
Admission Date/Time:
Exit Date:
Extension Request: Yes [ ] No [ ]

-- FICTIONAL DEMO RECORD - not a real patient --

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST
Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal
Member
Member Name: Ruth Doe
DOB: 11/02/1964
Member ID: OCH-73318504
Referring Provider
Facility: Sunridge Community Hospital (fictional)
Clinical
Primary Diagnosis: Diverticulosis of large intestine with bleeding
ICD-10: K57.31
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 10/21/2026
Requested Days: 14
Secondary Dx: Acute posthemorrhagic anemia (D62)
Plan Use Only
Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Page 3: CalAIM Community Supports authorization request

calaim_auth | orientation normal | legibility good

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Ruth Doe
DOB: 11/02/1964
Member ID: OCH-73318504

Referring Provider

Facility: Sunridge Community Hospital (fictional)

Clinical

Primary Diagnosis: Diverticulosis of large intestine with bleeding
ICD-10: K57.31
Member is experiencing homelessness **[X]**
Member at risk of hospitalization/re-hospitalization **[X]**
Requested Start Date: 10/21/2026
Requested Days: 14
Secondary Dx: Acute posthemorrhagic anemia (D62)

Plan Use Only

Status: Pending review

-- FICTIONAL DEMO RECORD - not a real patient --

Patient face sheet

Sunridge Community Hospital (fictional) - PATIENT FACE SHEET

Patient: Ruth Doe
MRN: SR-2207461
Account #: 99207461
DOB: 11/02/1964
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 10/17/2026 03:40
Admit Type: Inpatient
Unit/Bed: 3N-318
Attending: Anika Brandt, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 4: Patient face sheet

face_sheet | orientation normal | legibility good

Sunridge Community Hospital (fictional) - PATIENT FACE SHEET

Patient: Ruth Doe
MRN: SR-2207461
Account #: 99207461
DOB: 11/02/1964
Sex: Female
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
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Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
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Encounter

Admit Date/Time: 10/17/2026 03:40
Admit Type: Inpatient
Unit/Bed: 3N-318
Attending: Anika Brandt, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 1

HISTORY AND PHYSICAL
Date of Service: 10/17/2026
Attending: Anika Brandt, MD (Hospital Medicine)
Chief Complaint
Bright red blood per rectum x 1 day, lightheaded
HPI
61 y/o F experiencing homelessness (staying in a shelter bed at Example Shelter x 3 months) presents with 3 episodes of painless bright red blood per rectum and lightheadedness. Denies hematemesis or melena. Denies NSAID use. In ED: HR 112, BP 98/60, Hgb 6.8.
PMH
- Diverticulosis (per pt, colonoscopy years ago)
- Hypothyroidism
PSH
- None
Social History
Homeless x 3 months, shelter. Denies tobacco, alcohol or drug use.
Allergies
Penicillin (rash)

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 5: History and Physical - page 1

h_and_p | orientation normal | legibility good

HISTORY AND PHYSICAL

Date of Service: 10/17/2026
Attending: Anika Brandt, MD (Hospital Medicine)

Chief Complaint

Bright red blood per rectum x 1 day, lightheaded

HPI

61 y/o F experiencing homelessness (staying in a shelter bed at Example Shelter x 3 months) presents with 3 episodes of painless bright red blood per rectum and lightheadedness. Denies hematemesis or melena. Denies NSAID use. In ED: HR 112, BP 98/60, Hgb 6.8.

PMH

- Diverticulosis (per pt, colonoscopy years ago)
- Hypothyroidism

PSH

- None

Social History

Homeless x 3 months, shelter. Denies tobacco, alcohol or drug use.

Allergies

Penicillin (rash)

-- FICTIONAL DEMO RECORD - not a real patient --

History and Physical - page 2

Physical Exam
VS: T 98.2F, HR 112, BP 98/60, RR 18, SpO2 99% RA
Gen: pale, tired, AOx4
CV: tachycardic, regular
Abd: soft, non-tender, non-distended
Rectal: maroon and bright red blood, no masses
Psych: calm, cooperative
Assessment / Plan
61F with acute lower GI bleed, likely diverticular, with acute blood loss anemia.
1. LGIB - 2 large bore IVs, transfuse 2 units PRBC, serial Hgb q6h, GI consult for colonoscopy
2. Hold DVT chemoprophylaxis; SCDs
3. MRSA nares PCR, COVID per protocol
4. Dispo - CM consult; homeless
Isolation: None - standard precautions
Code status: Full
Electronically signed: Anika Brandt, MD 10/17/2026 05:15

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 6: History and Physical - page 2

h_and_p | orientation normal | legibility good

Physical Exam

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Isolation: None - standard precautions
Code status: Full
Electronically signed: Anika Brandt, MD 10/17/2026 05:15

-- FICTIONAL DEMO RECORD - not a real patient --

Gastroenterology consult and colonoscopy

GASTROENTEROLOGY CONSULT / PROCEDURE NOTE
Date: 10/18/2026 13:30
Consultant: Oren Takahashi, MD (Gastroenterology)
Procedure: Colonoscopy after bowel prep
Findings: Extensive sigmoid and descending colon diverticulosis. No active bleeding. No stigmata of recent hemorrhage requiring intervention. No masses, no colitis.
Upper endoscopy not indicated (no hematemesis, BUN not elevated).
Impression: Diverticular bleed, self-limited. No ulcer disease. No varices.
Recommendations: High fiber diet. Avoid NSAIDs. Outpatient GI follow-up in 4-6 weeks. OK for discharge from GI standpoint if Hgb stable.
Signed: Oren Takahashi, MD

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 7: Gastroenterology consult and colonoscopy

consult_note | orientation normal | legibility good

GASTROENTEROLOGY CONSULT / PROCEDURE NOTE

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Impression: Diverticular bleed, self-limited. No ulcer disease. No varices.
Recommendations: High fiber diet. Avoid NSAIDs. Outpatient GI follow-up in 4-6 weeks. OK for discharge from GI standpoint if Hgb stable.
Signed: Oren Takahashi, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Hospitalist progress note

HOSPITALIST PROGRESS NOTE – HD#5
Date: 10/21/2026 08:00
S: No further bleeding since 10/17 PM. Lightheadedness resolved. Tolerating diet.
O: T 98.1F HR 82 BP 122/74 RR 16 SpO2 99% RA
Hgb 9.1 (10/20) and 9.0 (10/21), stable on two checks; s/p 2 units PRBC 10/17.
No bowel movements with blood x 72h.
A/P:
- Diverticular LGIB, resolved. Hgb stable. GI follow-up 4-6 weeks.
- Acute blood loss anemia - start oral iron.
- Hypothyroidism - continue home levothyroxine.
- Medically stable for d/c to recuperative care today ~14:00.
Signed: Anika Brandt, MD

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 8: Hospitalist progress note

progress_note | orientation normal | legibility good

HOSPITALIST PROGRESS NOTE - HD#5

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S: No further bleeding since 10/17 PM. Lightheadedness resolved. Tolerating diet.
O: T 98.1F HR 82 BP 122/74 RR 16 SpO2 99% RA
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No bowel movements with blood x 72h.
A/P:

- Diverticular LGIB, resolved. Hgb stable. GI follow-up 4-6 weeks.
- Acute blood loss anemia - start oral iron.
- Hypothyroidism - continue home levothyroxine.
- Medically stable for d/c to recuperative care today ~14:00.

Signed: Anika Brandt, MD

-- FICTIONAL DEMO RECORD - not a real patient --

Case management and social work note

CASE MANAGEMENT / SOCIAL WORK NOTE
Date: 10/21/2026 09:00
Pt AOx4, pleasant and cooperative. No behavioral concerns per nursing.
Psychosocial screen: denies depression. Denies SI/HI. Denies auditory or visual hallucinations. No psychiatric history. No history of conservatorship or FSP.
Substance use: denies.
Not a registered sex offender per pt.
Pt agreeable to recuperative care.
GI follow-up: to be scheduled by GI clinic after d/c.
Signed: Gabriel Montoya, MSW

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 9: Case management and social work note

social_work | orientation normal | legibility good

CASE MANAGEMENT / SOCIAL WORK NOTE

Date: 10/21/2026 09:00
Pt AOx4, pleasant and cooperative. No behavioral concerns per nursing.
Psychosocial screen: denies depression. Denies SI/HI. Denies auditory or visual hallucinations. No psychiatric history. No history of conservatorship or FSP.
Substance use: denies.
Not a registered sex offender per pt.
Pt agreeable to recuperative care.
GI follow-up: to be scheduled by GI clinic after d/c.
Signed: Gabriel Montoya, MSW

-- FICTIONAL DEMO RECORD - not a real patient --

Vitals and labs

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
10/20 08:00 | 98.4 | 86 | 120/72 | 16 | 99% | RA
10/20 16:00 | 98.2 | 84 | 118/74 | 16 | 98% | RA
10/21 00:00 | 98.0 | 78 | 116/70 | 14 | 99% | RA
10/21 04:00 | 98.1 | 80 | 118/72 | 16 | 99% | RA
10/21 08:00 | 98.1 | 82 | 122/74 | 16 | 99% | RA

LABS

10/17 02:50 | Hgb 6.8 L | Plt 242 | BUN 14 | Cr 0.8 | INR 1.0
10/18 05:00 | Hgb 8.6 L (post 2 units PRBC)
10/20 05:00 | Hgb 9.1 L | Cr 0.7
10/21 05:00 | Hgb 9.0 L | TSH 2.1

MICROBIOLOGY

10/17 MRSA nares PCR: Negative
10/21 06:00 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/17 CXR: clear lungs

-- FICTIONAL DEMO RECORD - not a real patient --

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Page 10: Vitals and labs

vitals | orientation normal | legibility good

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
10/20 08:00 | 98.4 | 86 | 120/72 | 16 | 99% | RA
10/20 16:00 | 98.2 | 84 | 118/74 | 16 | 98% | RA
10/21 00:00 | 98.0 | 78 | 116/70 | 14 | 99% | RA
10/21 04:00 | 98.1 | 80 | 118/72 | 16 | 99% | RA
10/21 08:00 | 98.1 | 82 | 122/74 | 16 | 99% | RA

LABS

10/17 02:50 | Hgb 6.8 L | Plt 242 | BUN 14 | Cr 0.8 | INR 1.0
10/18 05:00 | Hgb 8.6 L (post 2 units PRBC)
10/20 05:00 | Hgb 9.1 L | Cr 0.7
10/21 05:00 | Hgb 9.0 L | TSH 2.1

MICROBIOLOGY

10/17 MRSA nares PCR: Negative
10/21 06:00 SARS-CoV-2 rapid antigen: Negative

IMAGING

10/17 CXR: clear lungs

-- FICTIONAL DEMO RECORD - not a real patient --